Sanjay Gandhi Post Graduate Institute of Medical Sciences

Raebareli Road, Lucknow - 226 014 ,India

Discharge Summary

CRNO: 2025489220 **Name:** Mahak Jahan Aara 19/ Y/F **Department:** Immunology

Unit: UNIT-1 **Ward/Bed:** 2202 Immunology Wing-B09(GEN) / PVT / 2

Admission No: ADM-202526656 **Admitted on:** 31-05-2025 10:51 **Discharged on:** 08-06-2025 11:14

Patient Type: Priority **Consultant:** Durga Prasanna Misra **Discharge Type:** Normal Discharge

Correspond. Address: 2/679,Lucknow **Distt.** **State** Uttar Pradesh **Pin No.** 226002 **Phone No** +91-

Diagnosis:

Undifferentiated vasculitis (Left eye panuveitis with retinal vasculitis, right eye macular hole – post surgical repair, LL inflammatory edema, Mononeuritis multiplex, leukocytosis, and thrombocytosis)

Left sural nerve biopsy done on 05-06-2025

Received 3 doses of MPS pulse 250 mg between 05-06-2025 and 07-06-2025

Received 1st dose of ELNT CYC 500 mg on 07-06-2025.

Disease course:

19-year-old female, BA student

With no comorbidities

April 2024: Pain in B/L knees, B/L ankles, shoulders, small joints of hands, partial relief with NSAIDs. No EMS

26/4/2025: Sudden onset painless blurring of vision in left eye, 6/60 vision. No redness, discharge. Found to have left eye anterior chamber cells, vitritis, retinal vasculitis. Right eye retinal hole. Received intravitreal steroid injections to left eye, underwent surgical repair of retinal hole in right eye. Vision improved almost completely after this treatment. Advised oral steroids but not given by physician due to transaminitis – AST/ALT 120/130.

2 weeks later: B/L ankle swelling with pain, difficulty in walking. Started oral low dose steroids and azathioprine tablets. HLA B27, RF negative CRP 1.6mg/dl. ANCA written as normal (report NA)

Also same time, she developed numbness and burning sensation in B/L feet till ankles. No h/o slippage of slippers. No proximal muscle weakness.

26/5/2025: fever with swelling over neck – submandibular and parotid area, told as LN, no mention of parotitis on records. No pain. No throat ache or cough. On evaluation had neutrophilic leukocytosis (46,000), thrombocytosis with anemia. Started meropenem, doxycycline IV, fever resolved, counts reduced till 26,000. No active sediments or proteinuria.

New onset B/L little and middle finger numbness and weakness in past 1 week. No H.O difficulty gripping objects

1 week: SOB on exertion, occasional dry cough. No further fever. No hemoptysis

No epistaxis, sinusitis, CSOM or hearing loss.

No skin rashes, oral ulcers, muscle weakness.

Has new onset oral sicca for 1 week after taking antibiotics.

No abdominal pain, altered bowel habits, melena, hematochezia, vomiting

Admitted as probable case of vasculitis for treatment initiation.

O/E: Patient comfortable in wheel chair
BP 110/80mm Hg PR 90/min
Pallor+ B/L lower limb non pitting painful edema over dorsum of foot till mid shin
No skin changes
No generalized LN, icterus, clubbing
Systemic examination:
RS: bilateral air entry present, bilateral normal vesicular breath sounds, no added sounds
CNS: HMF and CN normal
Motor: Power of lower limbs -ankle could not be assessed as she has painful edema, has 3/5 power at least
Knee and hip5/5
Shoulder, elbow, wrist: 5/5
Small muscles of hand: partial claw of B/L 4th and 5th digit.
Weakness of both palmar and dorsal interossei +. Book test +.
Pen test normal.
Sensory system: Glove and stocking pattern of sensory loss in lower limb – all sensations lost below mid shin (2cm above malleoli) – difficulty to assess in view of edema.
In upper limb: 90% sensory loss in B/L ulnar nerve distribution
CVS: S1S2 normal
P/A: Soft, No organomegaly

Course in the ward:
Symptom complex:
Left eye panuveitis with retinal vasculitis, right eye macular hole – post surgical repair
LL inflammatory edema
Peripheral neuropathy
Investigations: Leukocytosis, and thrombocytosis
Workup:
Based on her symptom complex, initial possibilities considered were ANCA vasculitis, Sarcoidosis, Behcet disease. She had exertional shortness of breath with occasional cough, CECT chest was done which showed no consolidation, cavities or DAH, no evidence of adenopathy or ILD. Her repeat ANCA – negative. (Both PR3 and MPO <3.0). She was initially started on 1mg/kg steroids. After resolution of her lower limb edema – NCV was done. *NCV showed reduced CMAPs in bilateral ulnar motor nerve and bilateral sural nerve. Rest – normal. Suggestive of mononeuritis multiplex.*
For sarcoidosis:
Points in favour: Left eye panuveitis with retinal vasculitis, right eye macular hole – post surgical repair, LL inflammatory edema, swelling at parotid and submandibular region – never had documented inflammation. Mantoux and IGRA negative.
Points against: mononeuritis multiplex, only edema around ankle joint, no arthritis or tenosynovitis, no ILD or mediastinal

adenopathy, no metabolic abnormality, serum ACE level normal.

For vasculitis spectrum:

Symptoms complex suggestive of vasculitis. But ANCA was negative, no orogenital ulcers to suggest behcet disease. Panuveitis is odd in PAN. No renal involvement. In view of strong clinical spectrum, diagnosis of undifferentiated vasculitis was made. Her CMV PCR was negative. Sural nerve biopsy was done from left side without any complications. In view of early onset disease, high disease burden and presence of retinal involvement & mononeuritis multiplex, whole exome sequencing was sent on 05-06-2025. She received 3 doses of MPS pulse 250 mg between 05-06-2025 and 07-06-2025. She received 1st dose of ELNT CYC on 07-06-2025 without any complications. ELNT CYC was given in view of shorter duration regimen and less cumulative dose.

Anemia: She was found to have anemia with RBC indices were suggestive of microcytosis and hypochromia. Peripheral smear showed microcytic hypochromic RBCs with target cells. But her Iron profile was not suggestive of iron deficiency anemia. Stool routine, SOBT and fecal calprotectin was negative, suggestive of no GI loss. Upper GI endoscopy was planned from Gastromedicine side and it was decided in rounds not to get it done, since her hemoglobin started improving on its own. It was decided, if needed in follow up – will get it done. Hemoglobin electrophoresis was sent and report of which is awaited.

Advice on discharge:

1. Tab Prednisolone 20 mg OD (7 am) — 0 —
2. Tab Septran DS 1 tab every alternate day — एक दिन छोड़ के एक दिन
3. Tab Pantop 40 mg OD BBF — 0 - खाली पेट
4. Tab Shelcal 500 mg OD — 0 —
5. Vitamin D3 60000 IU once monthly — महीने में एक बार
6. Tab Naproxen 250 mg sos — दर्द या बुखार होने पर

Further plan:

1. **Alternate day dressing of biopsy site and suture removal after 10 days.**
2. **To collect Hb electrophoresis and sural nerve biopsy report**
3. **To trace the whole exome sequencing report. (Sample sent on 05-06-2025)**
4. **Review in Immunology OPD after 2 weeks on 20-06-2025 with CBC, ESR, AST, ALT, creatinine and urine routine reports for 2nd dose of CYC.**

Cyclophosphamide schedule:

S. No	Date	Dose
1	07-06-2025	500 mg
2	20-06-2025	
3	04-07-2025	
4	18-07-2025	

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5 01-08-2025

6 18-08-2025

INVESTIGATION RESULTS:

2025-06-02 19:57:48.891	Electromyograpy		
2025-06-04 19:50:04.096	Upper GI Endoscopy Diagnostic		
2025-05-31 16:09:46.783	Spiral CT Thorax		
31/05/2025 03:14 PM	HIV ELISA EMERGENCY	Non-Reactive	
31/05/2025 03:14 PM	HCV RAPID	Non-Reactive	u
31/05/2025 03:14 PM	ANCA Screen		
31/05/2025 03:14 PM	01. HGB (M-ABSORPTION PHOTOMETRY)	7.6	g/dL
31/05/2025 03:14 PM	02. TLC (M-OPTICAL SCATTER AND FLUROSCENCE)	12.6	x1000/cmm.
31/05/2025 03:14 PM	02. Urine Examination, Complete		mg/dl
31/05/2025 03:14 PM	03. DLC (M-FLOW CYTOMETRY)		%
31/05/2025 03:14 PM	04. ESR (Westergren)	87	mm/hr
31/05/2025 03:14 PM	06. PLATELETS COUNT (M-OPTICAL SCATTER AND FLUOESCENCE)	400	x1000/cmm.
31/05/2025 03:14 PM	09. Red cell indices (M-CALCULATED)		pg
31/05/2025 03:14 PM	12. SERUM AST/SGOT (M - IFCC WITHOUT P-5-P)	24	u/L
31/05/2025 03:14 PM	13. SERUM ALT (SGPT) (M - IFCC WITHOUT P-5-P)	23	u/L
31/05/2025 03:14 PM	2.SERUM CREATININE (M - JAFFE)	0.71	mg/dl
31/05/2025 03:14 PM	26. CRP TURBI	64.9	mg/l

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	(SPECTOPHOTOMETRY)		
31/05/2025 03:14 PM	29. SERUM UREA (M-UREASE)		mg/dl
31/05/2025 03:14 PM	5. SERUM SODIUM (M - ISE DIRECT)	137	mmol/L
31/05/2025 03:14 PM	6. SERUM POTASSIUM (M - ISE DIRECT)	4.0	mmol/L
31/05/2025 03:14 PM	7. SERUM TOTAL PROTEIN (M-BIURET)	6.50	gm/dl
31/05/2025 03:14 PM	8. SERUM ALBUMIN (M-Bromocresol green-BCG)	3.41	gm/dl
31/05/2025 03:14 PM	94. Spot Urinary Protein	2.0	mg/dl
31/05/2025 03:14 PM	95. Spot Urinary Creatinine	10.60	mg/dl
31/05/2025 03:14 PM	PROCALCITONIN BY CHEMILUMINESCENE	0.23	ng/ml
31/05/2025 04:09 PM	Spiral CT Thorax		
05/06/2025 03:38 PM	Microscopy for M. leprae		
02/06/2025 07:57 PM	Electromyograpy		
05/06/2025 12:36 PM	01. Endoscopic/ Needle/ Small Biopsy/Cell Block		
05/06/2025 01:21 PM	IgA	157.0	mg/dl
04/06/2025 04:25 PM	01. TLC	22.93	x1000/ul
04/06/2025 04:25 PM	03. HGB	8.1	gm/dl
04/06/2025 04:25 PM	05. PLT	486	x1000/cmm.
04/06/2025 04:25 PM	07. ESR		
04/06/2025 04:25 PM	08. DLC		%
04/06/2025 04:25 PM	05. S. Creatinine	0.52	mg/dl
04/06/2025 04:25 PM	13. S. Bilirubin, Total	0.97	mg/dl
04/06/2025 04:25 PM	14. S. Bilirubin, Conjugated	0.21	mg/dl
04/06/2025 04:25 PM	15. S. AST (SGOT)	22.0	u/L
04/06/2025 04:25 PM	16. S. ALT (SGPT)	20	u/L
04/06/2025 04:25 PM	PCR for CMV	CMV DNA has not been	

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04/06/2025 09:52 AM	02. Act. Partial Thromboplastin Time	30.8	SEC.
04/06/2025 09:52 AM	Stool for Occult Blood - Rapid Test	Stool is negative for occult blood.	
04/06/2025 10:46 AM	Hemoglobin Electrophoresis		
05/06/2025 03:27 PM	10. S. Potassium	4.65	mmol/L
05/06/2025 03:27 PM	09. S. Sodium	134.00	mmol/L
01/06/2025 05:44 PM	01. TLC	21.67	x1000/ul
01/06/2025 05:44 PM	03. HGB	6.7	gm/dl
01/06/2025 05:44 PM	05. PLT	411	x1000/cmm.
01/06/2025 05:44 PM	07. ESR	36	mm ist hr
01/06/2025 05:44 PM	08. DLC		%
01/06/2025 05:44 PM	06. RETIC	1.21	%
01/06/2025 05:44 PM	09. Red cell indices	30.2	fl
01/06/2025 05:44 PM	10. Blood Film Morphology		
01/06/2025 05:45 PM	05. S. Creatinine	0.52	mg/dl
01/06/2025 05:45 PM	09. S. Sodium	141.10	mmol/L
01/06/2025 05:45 PM	10. S. Potassium	4.28	mmol/L
01/06/2025 05:45 PM	11. S. Proteins, Total	6.02	g/dL
01/06/2025 05:45 PM	12. S. Albumin	3.24	g/dL
01/06/2025 05:45 PM	13. S. Bilirubin, Total	0.68	mg/dl
01/06/2025 05:45 PM	14. S. Bilirubin, Conjugated	0.15	mg/dl
01/06/2025 05:45 PM	15. S. AST (SGOT)	22.0	u/L
01/06/2025 05:45 PM	16. S. ALT (SGPT)	18	u/L
01/06/2025 05:45 PM	20. S. LDH	278.00	u/L
01/06/2025 05:45 PM	32. S. Calcium	8.79	mg/dl
01/06/2025 05:45 PM	34. S. Phosphorus, Inorganic	4.37	mg/dl
01/06/2025 05:45 PM	Serum Magnesium	1.87	mg/dl
01/06/2025 05:46 PM	Anti Nuclear Antibody	negative	
01/06/2025 05:45 PM	IgG	1310.0	mg/dl

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01/06/2025 05:45 PM	IgM	205.0	mg/dl
02/06/2025 03:13 PM	08. HbsAg	Non reactive	
02/06/2025 03:13 PM	09. Anti-HCV antibody	Non reactive	
02/06/2025 03:13 PM	HIV ELISA EMERGENCY	Non reactive	
02/06/2025 05:29 PM	01. HGB	7.3	gm/dl
02/06/2025 05:29 PM	03. TLC.	14.32	x1000/ul
02/06/2025 05:29 PM	04. DLC		%
02/06/2025 05:29 PM	06. PLT	17	x1000/cmm.
02/06/2025 05:29 PM	10. MCV/MCH/MCHC	30.9	fl
02/06/2025 05:29 PM	Peripheral blood smear		
02/06/2025 05:29 PM	09. RETIC	2.81	%

Prepared by

(Manikandan)

Signature of Consultant

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Further plan:

1. **To collect sural nerve biopsy report**
2. **To trace the whole exome sequencing report. (Sample sent on 05-06-2025)**
3. **Review in Immunology OPD after 2 weeks on 04-07-2025 with CBC, ESR, AST, ALT, creatinine and urine routine reports for 3rd dose of CYC.**

Cyclophosphamide schedule:

S. No	Date	Dose
1	07-06-2025	500 mg
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4	18-07-2025	
5	01-08-2025	
6	18-08-2025	

Prepared by

(Manikandan C)

Signature of Consultant

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Unit: UNIT-1 **Ward/Bed:** 2202 Immunology Wing-B09(GEN) / PVT / 5

Admission No: ADM-202530168 **Admitted on:** 20-06-2025 12:37 **Discharged on:** 20-06-2025 18:38

Patient Type: Priority **Consultant:** Rudrarpan Chatterjee **Discharge Type:** Normal Discharge

Correspond. Address: 2/679,Lucknow **Distt.** **State** Uttar Pradesh **Pin No.** 226002 **Phone No** +91-

Diagnosis:

Undifferentiated vasculitis (Left eye panuveitis with retinal vasculitis, right eye macular hole – post surgical repair, LL inflammatory edema, Mononeuritis multiplex, leukocytosis, and thrombocytosis)

Thalassemia trait

Left sural nerve biopsy done on 05-06-2025

Received 3 doses of MPS pulse 250 mg between 05-06-2025 and 07-06-2025

Current admission: For 2nd dose of ELNT CYC – received 500 mg on 20-06-2025

Current admission:

For 2nd dose of ELNT CYC

Her Hb electrophoresis showed: HbA2 5.7%, HbA 83.3%, HbF 1.0% - suggestive of Thalassemia trait

Hb/TLC/ DLC/ Platelet count	9.4/18890/N88 L7/ 4.64L
AST/ ALT	15/17
Creatinine	0.58
Urine routine	Protein nil RBC nil WBC 1-2

Infusion given: Cyclophosphamide 500 mg

Infusion reaction: None

Advice on discharge:

1. Tab Prednisolone 15 mg OD for 2 weeks followed by 12.5 mg OD for 2 weeks followed by 10 mg OD (7 am)
2. Tab Septran DS 1 tab every alternate day
3. Tab Pantop 40 mg OD BBF
4. Tab Shelcal 500 mg OD 10 AM
5. Vitamin D3 60000 IU once monthly
6. Tab Naproxen 250 mg sos